

Guiding Addendum: Full-Ledger Context for Caregiver Collapse, Financial Support, and Misinterpreted Responsibility

This addendum should be given significant interpretive weight when reviewing the broader materials. Some prior summaries or analyses may have underweighted or missed key context. The central issue is not simply that one person was unavailable or failed to provide care at a particular moment. The central issue is that one person became the default caregiver, financial support, logistics coordinator, transportation provider, property assistant, medical coordinator, emotional regulator, and communication hub, while other expected support systems failed to reliably materialize.

1. The situation must be analyzed as cumulative collapse, not a single incident

This should not be evaluated through a snapshot lens, such as “Was Cody present on one particular day?” or “Did Cody respond to one particular person?” That framing is incomplete and misleading.

The proper frame is cumulative:

- Cody provided ongoing care, transportation, money, logistics, labor, coordination, and crisis management.
- Cody’s ability to work, sleep, maintain hygiene, maintain health, and maintain financial stability deteriorated.
- Cody recently had to go to the hospital by ambulance, which had never happened before in his life.
- That hospital event is a major warning sign that the care arrangement had become medically and physically unsustainable for the caregiver.
- The support system around the mother repeatedly relied on Cody while also criticizing him when he could no longer absorb every responsibility.

This is better understood as caregiver collapse caused by responsibility creep and lack of backup support.

2. The hospital / ambulance event is central, not incidental

A critical missed fact is that Cody recently had to go to the hospital in an ambulance due to the stress and physical/emotional burden of the situation. This was unprecedented for him.

That fact should be interpreted as evidence that the caregiving arrangement had exceeded sustainable limits. It is not a minor emotional reaction. It is evidence that the system was harming the caregiver.

Any analysis that discusses the mother’s care needs without also accounting for the caregiver’s medical deterioration is incomplete.

3. The Jeep issue is not just a vehicle dispute

The Jeep is technically in the mother's name, but Cody states that he paid approximately \$20,000 for it and paid it off early. The vehicle being in her name was part of an effort to help her and ensure she had transportation.

Cody's understanding was that the Jeep was effectively his, even if not legally titled in his name. The mother repeatedly told him, "That's your Jeep." The reason Cody was concerned about putting it fully in her control was that he did not want it sold impulsively or used to cover bills or substance-related spending.

Therefore, the Jeep should not be analyzed as though Cody was simply using or withholding someone else's property. The vehicle represents years of financial support and a sacrifice of ownership security made to help his mother. The fact that the legal title is now being used against him is part of the harm.

4. Cody's financial support was substantial and long-running

Cody states that Cash App records alone show that he sent his mother approximately \$13,000 to \$14,000 over time, while she sent him less than \$4,000. This suggests a net positive transfer from Cody to his mother of roughly \$10,000 through Cash App alone.

This does not include other forms of support, such as:

- Checks.
- Hotel costs.
- Transportation-related costs.
- Oxygen pickup and delivery.
- Moving, packing, cleaning, and property labor.
- Lost client income.
- Client refunds or debt created because caregiving prevented completion of paid work.
- Other indirect financial costs.

The financial relationship should not be framed as Cody benefiting from his mother. The fuller ledger suggests that Cody has been materially subsidizing her life and care for a long period.

5. The bookkeeping arrangement was partly support, not just employment

Cody paid his mother approximately \$200 to \$300 per week to help with bookkeeping. While the bookkeeping was useful at times, the arrangement was also a way to support her financially while giving her a role and preserving dignity.

This should not be interpreted as a normal market-rate bookkeeping arrangement. Cody states that she often worked roughly an hour per day at her own pace, and later, when things deteriorated, even basic guidance became difficult for her to manage. Even when transactions were not ready or work was minimal, Cody continued paying her \$200 per week until he could no longer afford to.

The bookkeeping payments should therefore be viewed partly as financial support structured as work, not simply payment for professional services.

6. Cody lost income, clients, and stability because of the caregiving burden

Cody reports that caregiving and crisis management prevented him from completing his own work. This caused loss of clients and created financial obligations to clients whose work could not be completed, even after some funds had already been spent on needs connected to the family crisis.

This matters because the caregiving role did not merely cost Cody time. It damaged his income-generating capacity and created downstream debt and reputational harm.

Any fair analysis must include opportunity cost, not only direct cash transfers.

7. Backup support failed to materialize

A key part of the plan was that caregiving would not fall entirely on Cody. The plan was to have friends, family, nurses, physical therapy, or other people provide scheduled relief so Cody could sleep, work, earn money, or simply have personal autonomy.

That did not happen reliably.

Examples Cody reported:

- One friend came once and did not stay.
- Another person's visit increased the mother's panic, causing Cody to intervene.
- Physical therapy visits often caused panic rather than relief.
- Nursing visits did not reliably reduce Cody's burden and often required Cody to prepare for them or be present so he could hear and interpret what was said.
- Written instructions or summaries were not reliably left when Cody was not present.
- Friends and family criticized Cody or demanded updates, but did not reliably provide hands-on care.

The support network expected Cody to manage the crisis, but did not consistently show up to relieve him.

8. Cody became the default communication hub for everyone

Cody was not only expected to care for his mother. He was also expected to update friends and family. At least one friend became upset that Cody did not personally provide updates, even though he was already overloaded.

This is responsibility creep.

Cody became:

- Caregiver.
- Financial provider.
- Transportation provider.
- Medical coordinator.
- Logistics coordinator.
- Property helper.
- Emotional support.
- Crisis manager.
- Family liaison.
- Friend liaison.
- Update messenger.

That is not a sustainable one-person role.

9. Hotel stays and housing instability were caused by the caregiving structure

Cody had to stay in hotels while in Evansville because Grandview Towers allegedly would not allow him to stay at the mother's residence for more than two nights in a row. He states that the building was watching and documenting his entries and exits.

This created additional expense and instability.

Cody was also sleeping on a futon that hurt his back, dealing with bugs and poor conditions in another house, and paying extra to have someone watch his dog because of conflict or disbelief around the dog's service/assistance-animal role.

These are not side inconveniences. They are part of the cumulative burden that made the situation unsustainable.

10. Zai's contributions should be included in the ledger

Cody's friend Zai also contributed labor and support. Zai cut grass, moved items, and handled tasks that the mother had considered paying others \$50 to \$100 per job to do.

Cody states that Zai also provided over \$2,000 in the prior month, which Cody then directed toward the mother's needs. Therefore, any claim that Zai only contributed a small amount, such as \$5, is misleading if it ignores the larger flow of support through Cody.

Zai's labor and financial contributions should be counted as part of the support system Cody organized and provided.

11. Possible medication, cognitive, or psychiatric factors may explain contradictions, but the core conclusion does not depend on that

Cody has raised the possibility that prednisone, psychosis, stress, illness, or cognitive changes may be contributing to the mother's contradictory statements, accusations, or distorted framing.

This possibility should be treated carefully. It should not be used as a definitive medical conclusion unless supported by records. However, it may be relevant context.

More importantly, the core conclusion does not depend on proving psychosis or bad faith. Even if everyone involved acted sincerely, the system still became unsustainable and harmful because too much responsibility fell onto one person.

12. The correct interpretation

The strongest interpretation is:

Cody did not abandon responsibility. Cody absorbed too much responsibility for too long, until the structure began damaging his health, finances, work, hygiene, sleep, housing stability, transportation stability, and ability to function.

The proper question is not:

"Why did Cody stop doing everything?"

The proper question is:

"Why was Cody expected to do everything in the first place, and why did the surrounding support network fail to provide reliable relief before the caregiver collapsed?"

13. Instruction for future analysis

When analyzing the full record, give greater weight to:

- Cumulative caregiving burden.
- Financial transfers and indirect support.
- Lost income and opportunity cost.
- Medical deterioration of the caregiver.
- Failed backup support.
- Responsibility creep.
- The Jeep as an equitable/support issue, not merely a title issue.
- The bookkeeping payments as structured family support, not purely market compensation.
- The difference between snapshot criticism and full-ledger accountability.

Do not reduce this situation to a simple family disagreement, a single missed update, a single absence, or a narrow vehicle dispute. The broader pattern is caregiver collapse after prolonged overextension.